جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : رضا ابو سريع مرسى حسن
المحافظة : الجيزه **الحي / المدينة :** الهرم **العنوان :** 20 ش سلام حسين
رقم البطاقة : 26905120100484 **رقم التليفون :** 01117135212 **تاريخ تسجيل الإستمارة :** 15-10-2026
العمر(تاريخ الميلاد) : 12-05-1969 **المهنة :** حاصل علي دبلوم

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام تصوير مقطعي للصدر والبطن والحوض بالصبغة [11129] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: رضا ابو سريع مرسى حسن
National ID: 26905120100484

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

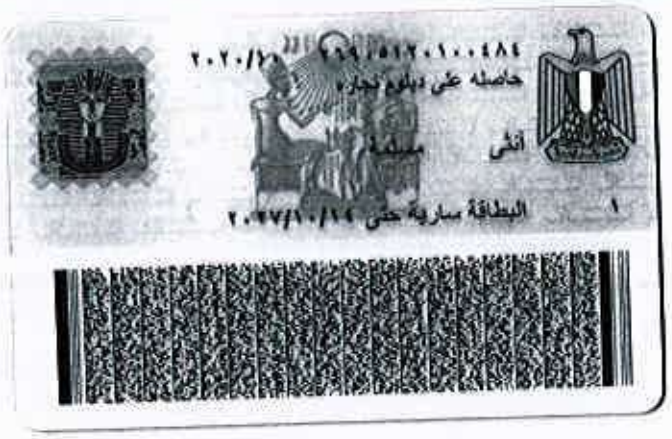
نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Contrast-enhanced CT of chest, abdomen and pelvis.**

التاريخ : **05-07-2026**	**الوقت :** **-- --:--**
	الطبيب المعالج : د/شيماء لاشين



حاصل

DMS
مزيد من الأبحاث
في موجات صوتية



مبـــادرة رئيس الجمهورية
لدعـم صحة المرأة المصرية

06/07/2026

MDT FORM

Name:	Rida Abusreaa Morsy Hassan	MRN:	2762
Age:	57 Y	ID:	26905120100484
Menopausal Sta	postm	Phone:	01117135212
NRS SCORE:	2	Marital Status:	
BMI:		Family History:	
CO Morbidity:		Reason Of MDT:	

Clinical Data:

BX:

ER: PR: HER2: KI67:

Sonomammography:

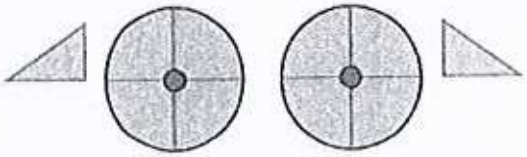
Mets Workup:

History:
postm left locally advanced left breast cancer met to lung er-ve pr-ve her2 3+ rec taxol and 6 herceptin dp rec 3 xeloda 2 tykerb cts 23/2/2026 pd of left breast mass pd of left ax ln pd of pulmonary nodulespt rec up to c3 d1 gemzar carboplatin
on exam left breast induration left axillary mass fixed mass 6x6 left loq ulcerative lesion for mdt bec of clinical progression approval of kadcyla pending from ngo

Recommendations:

MDT Outcome & Plan
for navelbine single agent

R L



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د/ هالة عزت	د/ أحمد ثروت				



Name: رضا ابوسريع مرسى حسن **MR. :** 2762	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient is a 57-year-old individual with a postmenopausal status who presents with a locally advanced left breast carcinoma that has metastasized to the lung. The disease course has been marked by progression despite multiple systemic therapies, including taxol and six cycles of trastuzumab, followed by sequential regimens of capecitabine (Xeloda) with lapatinib (Tykerb), and gemcitabine with carboplatin for two cycles. Most recently, the patient completed three cycles of ado-trastuzumab emtansine (Kadcyla) administered through a non-governmental organization, with the last dose given one week ago; shortly thereafter, the patient experienced bleeding from a fungating breast mass and reports subjective local pain.

تحريرا في ,

التاريخ :	23-09-2026	**الوقت :**	

الطبيب المعالج :

يعتمد ,

GUSTAVE ROUSSY
INTERNATIONAL
EGYPTE

٢٧٦٢

Name : Reda Abo Scree`a Morsi	**Date : 23-02-2026 11:41**
Branch : Portable	**Medical No. : GRI-99868**
Ref.Physician Prof : cap+c	**Ins.Company :**

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

A known case of left breast cancer, as compared to 22/11/2025, the current examination revealed

I- CHEST:

- Progressive course of left breast diffuse heterogenous parenchymal texture and size of the retro-areolar soft tissue mass lesion measures about 7x4 cm compared to 5.84x3 cm, progressive course overlying skin thickness measures about 1.96 cm compared to 1.44 cm
- Progressive course regarding size of ipsilateral enlarged axillary lymph nodes, the largest measures about 4.1x3.28 cm compared to 3.1x3 cm **for further sono-mammography.**
- Rather stationary course of right breast multifocal lesions _
- Progressive course regarding size and number of bilateral scattered pulmonary nodules &masses, the largest is seen in left lower lobar superior segment measures about 4.67x3.36 cm compared to 3x2.59 cm
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- The gall bladder is well distended, average wall thickness with no stones.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.

GUSTAVE ROUSSY
INTERNATIONAL
EGYPTE

Name : Reda Abo Seree'a Morsi	**Date : 23-02-2026 11:41**
Branch : Portable	**Medical No. : GRI-99868**
Ref.Physician Prof : cap+c	**Ins.Company :**

- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- **The examined skeleton** showed no worrisome lytic or sclerotic lesions.

Opinion:

- **Progressive course since the last CT examination**

Dr.Norhan Ramzy,MD

Dr.NOURHAN RAMZY

Dr.NOURHAN RAMZY

GUSTAVE ROUSSY
INTERNATIONAL

جوستاف روسي انترناشونال مصر

ECHO REPORT

NAME : رضا أبو سريع مرسي DATE: ٩ / ١١ /2025

ID NUMBER : ١٦٩٩/ ٢٥ SEX : ♂ AGE: ١

EF	60%	55-75%	PWT	0.8	0.6-1.1cm
LVEDD	4.8	3.5-5.8cm	AO.R	2.3	3-3.7cm
LVESD	2.5	2-4cm	LA	3.6	2-4cm
FS	29%	25-45%	RV		0.7-2.3com(supine)
SWT	0.9	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions Normal
LV Wall Thickness Normal
LV Diastolic Function GI
SWMA No RWMA
LA and Ao Root Dimensions Normal
Mitral Valve Scl, E trivial MR
Aortic Valve Scl
Tricuspid Valve trivial TR
Pulmonary Valve
Right Side and Systolic Function TAPSE: 2.3
Intracardiac Masses and Effusion

DR.

Patient Name	: السيدة / رضا ابو سريع عرجيه	Request No.	: 13009182
Patient No.	: 26905120100484	Request Date	: 31/05/2025
Claim No.	: 1961/25	Verification Date	: 12/06/2025
Sex	: Female	Financial Category	:
Age	: 68 Y	Department	: Out Patient
Referred Doc.	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

- Ki67: Positive in 40% of tumor cells.

* Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%
and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate and 3, strong). Total score from 0 to 8.
* Scoring system: "Her2"
Reference: ASCO-CAP Her2 Test Guideline Recommendations. 2013 IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.
IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of
the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signatures
Dr. Walaa Ebrahem Gharieb.
Dr. Heba Hussein.

Verified by :
Dr.Duaa Mohamed

Patient Name	: الاستاذه / رضا ابو سريع مرسى	Request No.	: 13009182
Patient No.	: 26905120100484	Request Date	: 31/05/2025
Path No.	: 1961/25	Verification Date	: 12/06/2025
Sex	: Female	Financial Category	: اورنج دار السلام - مباشره
Age	: 56 Y	Department	: Out Patient
Referring Doc.	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Left breast 5 O'clock mass.
Nature of Specimen: Tru-cut biopsy, collected at 29/05/2025.
Gross Examination:
- Multiple grey white tissue cores up to 1 cm long. Totally submitted.

Technique:
- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against: ER, PR, Her2, ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic Examination:
- Histological type: Invasive duct carcinoma.
- Grading/differentiation: nuclear pleomorphism+ mitosis): Grade III (3+3+2).
- DCIS: Not detected.
- Angiolymphasion: Not detected.
- Perineural invasion: Not detected.

IHC:
- ER: Negative nuclear reaction in tumor cells (score 0/8).
- PR: Weak nuclear reaction in 2% of tumor cells (score 3/8).
- Her2: Strong complete membrane reaction in more than 10 % of tumor cells.
- Ki67: 60% nuclear reaction of tumor cells.

Conclusion:
- Left breast mass, Tru-cut biopsy:
- Invasive duct carcinoma, Grade III.

IHC:
- ER: Negative (score 0/8).
- PR: Positive (score 3/8).
- Her2: Positive (score 3+).

Verified by :
Dr.Duaa Mohamed

Printing Date : 15/06/2025 10:46:15 Page 1 of 2 Printing By : Mrs.Mayada Hegazy

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

جوستاف روسي

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

البنت المصرية هي صحة مصر

MDT FORM 30 / 4 / 2025 G

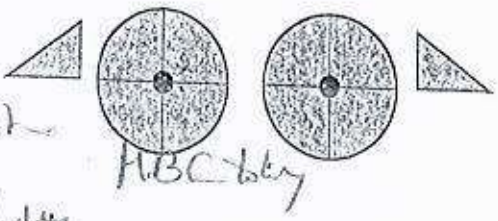
Name		MRN	1799/C.C5
		ID	٢٩٠٥١٢٠١٠٤٨٢
Age	58Y.	Phone	01117135212
Menopausal status	postmeno	Marital status	
NRS SCORE		Family history	
BMI		REASON OF MDT	
CO morbidity			

Clinical Data

BX: IDC III

ER:	PR:	HER 2:	Ki67:
-ve	3+	3+	

R PS: 1 L



TNM

Sonomammography: Post Oleft Breast CT4d, N2, M1 Lung

Mets Workup: Int CT:- Bilat. pulm Nodules Lt 2-8x2

pr Nac 4 Cycle. Taxol + 4 Herp

Recommendations: Local RD, Left Breast, Rt Axilla ↑sy left axillLN → Left LOR (6x6 -) mets obt for Lyst

MDT OUTCOME & PLAN: + grade 3 pN

٣٠/٤/٢٠٢٥ ف. MDT after surgery Xeloda 1111625 Mamm-Ms Ind

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

MDT Form
MDT FORM

GUSTAVE ROUSSY
INTERNATIONAL

Patient Name: Reda Abo Seree'a Morsi Hassan	Date : 22/11/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 56 Year(s)
Medical ID: 1699/2025	Pat. ID: 99868

MULTISCLICE C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS AFTER I.V. CONTRAST INJECTION REVEALED:

In reviewing the previous study report dated 20-7-2025, the current study showing:

I- CHEST:

- Left LIQ breas mass measures 5.4 x5.9cm with diffuse parecnhymaous edematous changes and overlying skin thickneing.
- Ipsilateral pathologically enlarged axillary lymph nodes,largest measuring about3.4x3.8cm
- Right breat mass wirh fine calcificatio oblong shape with regular border 3.8 x1.5 cm
- Bilateral multiple scattered pulmonary nodules, the largest seen at left lower lobe , the largest measures 2.8 x2.6 cm
- No sizable hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Bilateral small elatstofibroma dorsi,
- No gross cardiomegaly.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No focal lesion or dilated biliary radicles.
- Normal C.T appearance of the spleen, both kidneys, adrenal glands, pancreas, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No ascites.

Opinion:

GUSTAVE ROUSSY
INTERNATIONAL

Patient Name: Reda Abo Seree'a Morsi Hassan	Date : 22/11/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 56 Year(s)
Medical ID: 1699/2025	Pat. ID: 99868

Reported case of left breast cancer showing :

- **Left breast mass with pathological axillary lymph nodes.**
- **Right breast mass for sonomamography correlation.**
- **Bilateral pulmonary nodules likely metastasis**

Dr. Nourhan Mohamed,MD

Signature Dr.Mohamed Hassan
24/11/2025 5:46 PM

Patient's Name:	**Reda Abu Saree Mursi**
Date:	**2026-08-23**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

MULTISLICE CT EXAMINATIUON OF THE CHEST ,ABDOMEN AND PELVIS WITH CONTRAST ADMINISTRATION REVEALED:-

- An irregular-shaped enhancing density is still seen in the axillary tail of the left breast ,with left axillary lymph node and each measures about 6.1x3.7cm and 4.5x2.5 cm in dimensions respectively. Another enhancing lesion is also seen in the UIQ of the left breast and measures about 14x13mm in dimensions. A third lesion is also seen in the LIQ of the left breast with pop-corn calcifications noted herein with thickened trabeculations of the left mammary parenchyma and the overlying skin is diffusely thickened as well.
- An enhancing well-defined nodule is seen at 3 O'clock position of the right breast and measures about 15x12mm in dimensions.Another oval shaped solid mass is seen in the LOQ of the right breast with foci of calcifications noted inside and measures about 36x20mm in dimensions.
- Hyperinflated both lungs.
- Increased the size /number of the multiple inummerable subpleural /central enhancing pulmonary solid nodules scattering in both lung fields with the largest one seen in the anterior segment of right upper pulmonary lobe and measures about 91x41 mm in dimensions.
- Linear atelectatic bands are seen in the lingula and in both lower pulmonary lobes with thickened interstitium also noted herein.
- No patchy areas of consolidation or foci calcifications noted.
- Enhancing pretracheal lymph nodes are still seen with no appreciable changes as regards to their size.
- Patent central airways.
- No pleural or pericardial effusion.
- Average size of the heart with normal CT of its big vessels.
- Newly-developed tiny foci of osteosclerosis scattering the scanned lower dorsal and multiple lumber levels.
- Average sized liver,measuring about 14.6 cm, showing a homogenous parenchymal attenuation and enhancement pattern.
- No abnormally-enhancing hepatic focal lesions or dilated biliary radicles.
- Patent PV.
- Average sized spleen,measuring about 10.8 cm. No focal lesions seen inside.
- Thickened left adrenal gland is still seen.
- Normal CT appearance of the pancreas, functioning both kidneys, right suprarenal gland, aorta and IVC.
- No sizable abdominal or pelvic lymphadenopathy.
- No ascites.
- No intra-vesical stones,solid masses ,wall thickening or diverticulosis seen.
- A tiny left ovarian cyst is still seen and measures about 24x20mm in dimensions.
- Cuts taken in the rest of the pelvis are unremarkable.

IMPRESSIONS:

A status follow up and compared with the previous study done at 07/06/2026, the current study shows: still

- **An irregular-shaped highly-suspicious enhancing neoplastic density (BIRADS-5) still seen in the axillary tail of the left breast ,with left axillary metastatic lymphadenopathy . Another enhancing lesion seen in the UIQ of the left breast and a third lesion also seen in**

Patient's Name:	**Reda Abu Saree Mursi**
Date:	**2026-08-23**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

the LIQ of the left breast with pop-corn calcifications noted herein with thickened trabeculations of the left mammary parenchyma and the overlying skin is diffusely thickened as well.

- An enhancing well-defined nodule still seen at 3 O'clock position of the right breast and another oval shaped solid mass in the LOQ of the right breast with foci of calcifications noted inside. For dynamic MRI of the breasts and /or biopsy.
- Hyperinflated both lungs with multiple variable sized inummerable metastatic subpleural /central enhancing pulmonary solid nodules showing increased size and number,denoting a progressive course,with linear atelectatic bands in the lingula and in both lower pulmonary lobes , thickened interstitium also noted herein , enhancing pretracheal metastatic lymphadenopathy and metastatic thickened left adrenal gland are still seen,denoting a stable course and newly-developed tiny osteosclerotic metastatic bony lesions scattering in the lower dorsal and multiple lumber levels.
- A tiny left ovarian cyst.

Prof/Hanan El-Ahwal.

Dr.Hanan Alahwel

Dr.Hanan Alahwel

الهيئة العامة للمستشفيات والمعاهد التعليمية
مستشفى احمد ماهر التعليمى
The General Organization for Teaching Hospitals
and Institutes
Ahmad Maher Teaching Hospital

To whom it may concern

Mrs./ Mr ..... 60 years old

female/ male patient presented with diagnosis .....

..... M.B.C. to lung .....

She/ He was referred to us for (radical / adjuvant / Neoadjuvant / palliative) Radiotherapy

She/ He has received (3D/ IMRT) radiotherapy to.....

..... fungating lt. breast .....

.....

At Dose : 20 Gy/ 5 fx/ 1 w .

Additional (Boost) ; yes/ no at dose .....Gy/.....fx/.....w

(sequential / concomitant)

Started: 20/7/2026

Ended: 27/7/2026. With good tolerance .

Head of department
Dr. Hany William

E-mail: Amaher.gothi.com ت : 23913366 -23911838- ف :23914455 3411شارع بور سعيد – باب الخلق – القاهرة

Patient's Name:	**Reda Abu Saree Mursi**
Date:	**2026-08-23**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

MULTISLICE CT EXAMINATIUON OF THE CHEST ,ABDOMEN AND PELVIS WITH CONTRAST ADMINISTRATION REVEALED:-

- An irregular-shaped enhancing density is still seen in the axillary tail of the left breast ,with left axillary lymph node and each measures about 6.1x3.7cm and 4.5x2.5 cm in dimensions respectively. Another enhancing lesion is also seen in the UIQ of the left breast and measures about 14x13mm in dimensions. A third lesion is also seen in the LIQ of the left breast with pop-corn calcifications noted herein with thickened trabeculations of the left mammary parenchyma and the overlying skin is diffusely thickened as well.
- An enhancing well-defined nodule is seen at 3 O'clock position of the right breast and measures about 15x12mm in dimensions.Another oval shaped solid mass is seen in the LOQ of the right breast with foci of calcifications noted inside and measures about 36x20mm in dimensions.
- Hyperinflated both lungs.
- Increased the size /number of the multiple inummerable subpleural /central enhancing pulmonary solid nodules scattering in both lung fields with the largest one seen in the anterior segment of right upper pulmonary lobe and measures about 91x41 mm in dimensions.
- Linear atelectatic bands are seen in the lingula and in both lower pulmonary lobes with thickened interstitium also noted herein.
- No patchy areas of consolidation or foci calcifications noted.
- Enhancing pretracheal lymph nodes are still seen with no appreciable changes as regards to their size.
- Patent central airways.
- No pleural or pericardial effusion.
- Average size of the heart with normal CT of its big vessels.
- Newly-developed tiny foci of osteosclerosis scattering the scanned lower dorsal and multiple lumber levels.
- Average sized liver,measuring about 14.6 cm, showing a homogenous parenchymal attenuation and enhancement pattern.
- No abnormally-enhancing hepatic focal lesions or dilated biliary radicles.
- Patent PV.
- Average sized spleen,measuring about 10.8 cm. No focal lesions seen inside.
- Thickened left adrenal gland is still seen.
- Normal CT appearance of the pancreas, functioning both kidneys, right suprarenal gland, aorta and IVC.
- No sizable abdominal or pelvic lymphadenopathy.
- No ascites.
- No intra-vesical stones,solid masses ,wall thickening or diverticulosis seen.
- A tiny left ovarian cyst is still seen and measures about 24x20mm in dimensions.
- Cuts taken in the rest of the pelvis are unremarkable.

IMPRESSIONS:

A status follow up and compared with the previous study done at 07/06/2026, the current study shows: still

- **An irregular-shaped highly-suspicious enhancing neoplastic density (BIRADS-5) still seen in the axillary tail of the left breast ,with left axillary metastatic lymphadenopathy . Another enhancing lesion seen in the UIQ of the left breast and a third lesion also seen in**

Patient's Name:	**Reda Abu Saree Mursi**
Date:	**2026-08-23**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

the LIQ of the left breast with pop-corn calcifications noted herein with thickened trabeculations of the left mammary parenchyma and the overlying skin is diffusely thickened as well.

- An enhancing well-defined nodule still seen at 3 O'clock position of the right breast and another oval shaped solid mass in the LOQ of the right breast with foci of calcifications noted inside. For dynamic MRI of the breasts and /or biopsy.
- Hyperinflated both lungs with multiple variable sized inummerable metastatic subpleural /central enhancing pulmonary solid nodules showing increased size and number,denoting a progressive course,with linear atelectatic bands in the lingula and in both lower pulmonary lobes , thickened interstitium also noted herein , enhancing pretracheal metastatic lymphadenopathy and metastatic thickened left adrenal gland are still seen,denoting a stable course and newly-developed tiny osteosclerotic metastatic bony lesions scattering in the lower dorsal and multiple lumber levels.
- A tiny left ovarian cyst.

Prof/Hanan El-Ahwal.

Dr.Hanan Alahwel

Dr.Hanan Alahwel

الهيئة العامة للمستشفيات والمعاهد التعليمية
مستشفى احمد ماهر التعليمى
The General Organization for Teaching Hospitals
and Institutes
Ahmad Maher Teaching Hospital

To whom it may concern

Mrs./ Mr ..... 60 years old

female/ male patient presented with diagnosis .....

..... M.B.C. to lung .....

She/ He was referred to us for (radical / adjuvant / Neoadjuvant / palliative) Radiotherapy

She/ He has received (3D/ IMRT) radiotherapy to.....

..... fungating lt. breast .....

.....

At Dose : 20 Gy/ 5 fx/ 1 w .

Additional (Boost) ; yes/ no at dose .....Gy/.....fx/.....w

(sequential / concomitant)

Started: 20/7/2026

Ended: 27/7/2026. With good tolerance .

Head of department
Dr. Hany William

E-mail: Amaher.gothi.com ت : 23913366 -23911838- ف :23914455 3411شارع بور سعيد – باب الخلق – القاهرة

Patient's Name:	**Reda Abu Saree Mursi**
Date:	**2026-08-23**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

MULTISLICE CT EXAMINATIUON OF THE CHEST ,ABDOMEN AND PELVIS WITH CONTRAST ADMINISTRATION REVEALED:-

- An irregular-shaped enhancing density is still seen in the axillary tail of the left breast ,with left axillary lymph node and each measures about 6.1x3.7cm and 4.5x2.5 cm in dimensions respectively. Another enhancing lesion is also seen in the UIQ of the left breast and measures about 14x13mm in dimensions. A third lesion is also seen in the LIQ of the left breast with pop-corn calcifications noted herein with thickened trabeculations of the left mammary parenchyma and the overlying skin is diffusely thickened as well.
- An enhancing well-defined nodule is seen at 3 O'clock position of the right breast and measures about 15x12mm in dimensions.Another oval shaped solid mass is seen in the LOQ of the right breast with foci of calcifications noted inside and measures about 36x20mm in dimensions.
- Hyperinflated both lungs.
- Increased the size /number of the multiple inummerable subpleural /central enhancing pulmonary solid nodules scattering in both lung fields with the largest one seen in the anterior segment of right upper pulmonary lobe and measures about 91x41 mm in dimensions.
- Linear atelectatic bands are seen in the lingula and in both lower pulmonary lobes with thickened interstitium also noted herein.
- No patchy areas of consolidation or foci calcifications noted.
- Enhancing pretracheal lymph nodes are still seen with no appreciable changes as regards to their size.
- Patent central airways.
- No pleural or pericardial effusion.
- Average size of the heart with normal CT of its big vessels.
- Newly-developed tiny foci of osteosclerosis scattering the scanned lower dorsal and multiple lumber levels.
- Average sized liver,measuring about 14.6 cm, showing a homogenous parenchymal attenuation and enhancement pattern.
- No abnormally-enhancing hepatic focal lesions or dilated biliary radicles.
- Patent PV.
- Average sized spleen,measuring about 10.8 cm. No focal lesions seen inside.
- Thickened left adrenal gland is still seen.
- Normal CT appearance of the pancreas, functioning both kidneys, right suprarenal gland, aorta and IVC.
- No sizable abdominal or pelvic lymphadenopathy.
- No ascites.
- No intra-vesical stones,solid masses ,wall thickening or diverticulosis seen.
- A tiny left ovarian cyst is still seen and measures about 24x20mm in dimensions.
- Cuts taken in the rest of the pelvis are unremarkable.

IMPRESSIONS:

A status follow up and compared with the previous study done at 07/06/2026, the current study shows: still

- **An irregular-shaped highly-suspicious enhancing neoplastic density (BIRADS-5) still seen in the axillary tail of the left breast ,with left axillary metastatic lymphadenopathy . Another enhancing lesion seen in the UIQ of the left breast and a third lesion also seen in**

Patient's Name:	**Reda Abu Saree Mursi**
Date:	**2026-08-23**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

the LIQ of the left breast with pop-corn calcifications noted herein with thickened trabeculations of the left mammary parenchyma and the overlying skin is diffusely thickened as well.

- An enhancing well-defined nodule still seen at 3 O'clock position of the right breast and another oval shaped solid mass in the LOQ of the right breast with foci of calcifications noted inside. For dynamic MRI of the breasts and /or biopsy.
- Hyperinflated both lungs with multiple variable sized inummerable metastatic subpleural /central enhancing pulmonary solid nodules showing increased size and number,denoting a progressive course,with linear atelectatic bands in the lingula and in both lower pulmonary lobes , thickened interstitium also noted herein , enhancing pretracheal metastatic lymphadenopathy and metastatic thickened left adrenal gland are still seen,denoting a stable course and newly-developed tiny osteosclerotic metastatic bony lesions scattering in the lower dorsal and multiple lumber levels.
- A tiny left ovarian cyst.

Prof/Hanan El-Ahwal.

Dr.Hanan Alahwel

Dr.Hanan Alahwel

الهيئة العامة للمستشفيات والمعاهد التعليمية
مستشفى احمد ماهر التعليمى
The General Organization for Teaching Hospitals
and Institutes
Ahmad Maher Teaching Hospital

To whom it may concern

Mrs./ Mr ..... 60 years old

female/ male patient presented with diagnosis .....

..... M.B.C. to lung .....

She/ He was referred to us for (radical / adjuvant / Neoadjuvant / palliative) Radiotherapy

She/ He has received (3D/ IMRT) radiotherapy to.....

..... fungating lt. breast .....

.....

At Dose : 20 Gy/ 5 fx/ 1 w .

Additional (Boost) ; yes/ no at dose .....Gy/.....fx/.....w

(sequential / concomitant)

Started: 20/7/2026

Ended: 27/7/2026. With good tolerance .

Head of department
Dr. Hany William

E-mail: Amaher.gothi.com ت : 23913366 -23911838- ف :23914455 3411شارع بور سعيد – باب الخلق – القاهرة

Patient's Name:	**Reda Abu Saree Mursi**
Date:	**2026-08-23**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

MULTISLICE CT EXAMINATIUON OF THE CHEST ,ABDOMEN AND PELVIS WITH CONTRAST ADMINISTRATION REVEALED:-

- An irregular-shaped enhancing density is still seen in the axillary tail of the left breast ,with left axillary lymph node and each measures about 6.1x3.7cm and 4.5x2.5 cm in dimensions respectively. Another enhancing lesion is also seen in the UIQ of the left breast and measures about 14x13mm in dimensions. A third lesion is also seen in the LIQ of the left breast with pop-corn calcifications noted herein with thickened trabeculations of the left mammary parenchyma and the overlying skin is diffusely thickened as well.
- An enhancing well-defined nodule is seen at 3 O'clock position of the right breast and measures about 15x12mm in dimensions.Another oval shaped solid mass is seen in the LOQ of the right breast with foci of calcifications noted inside and measures about 36x20mm in dimensions.
- Hyperinflated both lungs.
- Increased the size /number of the multiple inummerable subpleural /central enhancing pulmonary solid nodules scattering in both lung fields with the largest one seen in the anterior segment of right upper pulmonary lobe and measures about 91x41 mm in dimensions.
- Linear atelectatic bands are seen in the lingula and in both lower pulmonary lobes with thickened interstitium also noted herein.
- No patchy areas of consolidation or foci calcifications noted.
- Enhancing pretracheal lymph nodes are still seen with no appreciable changes as regards to their size.
- Patent central airways.
- No pleural or pericardial effusion.
- Average size of the heart with normal CT of its big vessels.
- Newly-developed tiny foci of osteosclerosis scattering the scanned lower dorsal and multiple lumber levels.
- Average sized liver,measuring about 14.6 cm, showing a homogenous parenchymal attenuation and enhancement pattern.
- No abnormally-enhancing hepatic focal lesions or dilated biliary radicles.
- Patent PV.
- Average sized spleen,measuring about 10.8 cm. No focal lesions seen inside.
- Thickened left adrenal gland is still seen.
- Normal CT appearance of the pancreas, functioning both kidneys, right suprarenal gland, aorta and IVC.
- No sizable abdominal or pelvic lymphadenopathy.
- No ascites.
- No intra-vesical stones,solid masses ,wall thickening or diverticulosis seen.
- A tiny left ovarian cyst is still seen and measures about 24x20mm in dimensions.
- Cuts taken in the rest of the pelvis are unremarkable.

IMPRESSIONS:

A status follow up and compared with the previous study done at 07/06/2026, the current study shows: still

- **An irregular-shaped highly-suspicious enhancing neoplastic density (BIRADS-5) still seen in the axillary tail of the left breast ,with left axillary metastatic lymphadenopathy . Another enhancing lesion seen in the UIQ of the left breast and a third lesion also seen in**

Patient's Name:	**Reda Abu Saree Mursi**
Date:	**2026-08-23**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

the LIQ of the left breast with pop-corn calcifications noted herein with thickened trabeculations of the left mammary parenchyma and the overlying skin is diffusely thickened as well.

- An enhancing well-defined nodule still seen at 3 O'clock position of the right breast and another oval shaped solid mass in the LOQ of the right breast with foci of calcifications noted inside. For dynamic MRI of the breasts and /or biopsy.
- Hyperinflated both lungs with multiple variable sized inummerable metastatic subpleural /central enhancing pulmonary solid nodules showing increased size and number,denoting a progressive course,with linear atelectatic bands in the lingula and in both lower pulmonary lobes , thickened interstitium also noted herein , enhancing pretracheal metastatic lymphadenopathy and metastatic thickened left adrenal gland are still seen,denoting a stable course and newly-developed tiny osteosclerotic metastatic bony lesions scattering in the lower dorsal and multiple lumber levels.
- A tiny left ovarian cyst.

Prof/Hanan El-Ahwal.

Dr.Hanan Alahwel

Dr.Hanan Alahwel

الهيئة العامة للمستشفيات والمعاهد التعليمية
مستشفى احمد ماهر التعليمى
The General Organization for Teaching Hospitals
and Institutes
Ahmad Maher Teaching Hospital

To whom it may concern

Mrs./ Mr ..... 60 years old

female/ male patient presented with diagnosis .....

..... M.B.C. to lung .....

She/ He was referred to us for (radical / adjuvant / Neoadjuvant / palliative) Radiotherapy

She/ He has received (3D/ IMRT) radiotherapy to.....

..... fungating lt. breast .....

.....

At Dose : 20 Gy/ 5 fx/ 1 w .

Additional (Boost) ; yes/ no at dose .....Gy/.....fx/.....w

(sequential / concomitant)

Started: 20/7/2026

Ended: 27/7/2026. With good tolerance .

Head of department
Dr. Hany William

E-mail: Amaher.gothi.com ت : 23913366 -23911838- ف :23914455 3411شارع بور سعيد – باب الخلق – القاهرة